**LUITPARIA CANCER INSTITUTE**
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Reg. No. AS/GHY/HOSP/2009/0076

# DISCHARGE SUMMARY

| | | | |
|---|---|---|---|
| Name: | **Mr. BHUPEN KALITA** | IP No.: | **LCI/IP/26/2214** |
| Age / Sex: | 63 Y / M | Department: | Surgical Oncology (Unit II) |
| Consultant: | Dr. Dipankar Saikia | Ward / Bed: | SICU, then Ward 4, Bed 12 |

## Diagnosis

Carcinoma mid-thoracic oesophagus, squamous cell carcinoma, post neoadjuvant chemoradiation.

## Summary of admission

Mr. Kalita was admitted on 25th May 2026 for planned surgery after completing neoadjuvant chemoradiation. He underwent minimally invasive McKeown oesophagectomy with feeding jejunostomy on 26th May 2026 under general anaesthesia. He was extubated on post-operative day (POD) 1 and shifted to the ward on POD 3. Jejunostomy feeds were started on POD 1 and built up to the full requirement by POD 4. The right chest drain was removed on POD 5.

An oral contrast (gastrografin) swallow study on 01/06/2026 showed no anastomotic leak and normal conduit emptying; oral liquids were started thereafter and advanced to a semi-solid diet. Mild hoarseness of voice noted on POD 2 is improving; ENT review showed left vocal cord paresis and voice therapy was advised.

He was discharged on 5th June 2026 in a stable condition, tolerating semi-solids, with the feeding jejunostomy tube in place.

## Investigations at discharge

Hb 10.8 g/dL, TLC 8,900 /cu.mm, serum albumin 3.1 g/dL. Chest X-ray after drain removal: lungs fully expanded.

## Pending

**Final HPE report of the resected specimen is awaited.**



SYNTHETIC DOCUMENT — FOR DEMONSTRATION ONLY

## Medications on discharge

| Medicine | Dose / route | When | Days |
|---|---|---|---|
| Syp. Paracetamol 250 mg/5 mL | 15 mL orally | TDS, if pain | 5 |
| Tab. Pantoprazole 40 mg | 1 tab, dissolved, via FJ | OD morning | 30 |
| Tab. Metoclopramide 10 mg | 1 tab orally | TDS, before meals | 10 |
| Inj. Enoxaparin 40 mg | Subcutaneous | OD | 14 |
| Chlorhexidine 0.2 % mouthwash | 10 mL gargle | TDS | 7 |

## Diet and FJ care

Semi-solid oral diet in small frequent meals; sit upright for 30 minutes after eating. FJ feeds: 1.5 kcal/mL polymeric feed, 200 mL every 3 hours (6 feeds a day); flush with 30 mL water before and after each feed. Sleep with the head end raised.

## Follow-up

Review in Surgical Oncology OPD on 12/06/2026 with the HPE report; medical and radiation oncology review thereafter for the adjuvant plan. Report immediately for fever, neck swelling, breathlessness or displacement of the FJ tube.

**Dr. Dipankar Saikia**
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*Consultant Surgical Oncologist*



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